Clinical Management Handbook

Condition-Specific Pathways and Therapeutic Guidance

Prepared from Clinical_Guidelines_Compact and workbook clinical sheets

This handbook is written for clinical education and bedside decision support. It presents each condition in formal
prose, with emphasis on clinical assessment, pathway differentiation, and treatment selection in routine practice.

Condition 1: PV BLEEDING

Patients presenting with this condition should first undergo a structured clinical assessment that establishes physiological stability, severity, and immediate risk. Common presentations documented in the source material include Per Vaginal Bleeding. Important clinical features include with signd of shock(Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg; NO Local bleeding lesions; WITHOUT signs of shock(Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg & Hemodynamically stable. Initial triage should prioritize early identification of shock, respiratory compromise, neurological deterioration, or obstetric and surgical emergencies where relevant.

Diagnostic evaluation should proceed in a focused sequence that supports pathway selection and timely intervention. Core investigations described in the clinical sheets include FHG + Pelvic U/S as you continue fluid resuscitation; FHG + OBS U/S as you continue fluid resuscitation; FHG + Pelvic U/S. Interpretive patterns used to refine diagnosis include Normal FHG; Granulocytosis; Thrombocytopenia; Hb < 10 g/dl. Where diagnostic uncertainty remains, additional tests may be considered, including Hormonal profile, U/E/C,; Hormonal profile, U/E/C, + LFTs; Hormonal profile, U/E/C, Coagulation profile + PBF. This staged approach helps clinicians distinguish severe pathways from lower-risk pathways and supports safer escalation decisions.

Pathway-oriented diagnosis should then be articulated clearly in the clinical record. Documented diagnostic formulations include Probable dx: SHOCK with Abnormal Uterine Bleeding ? Cause; Probable dx: SHOCK with Abnormal Uterine Bleeding ? Cause , ITP, VWD,; Probable dx: SHOCK with Abnormal Uterine Bleeding ? Cause, Anemia; Probable dx: Abortion with SHOCK; Probable dx: Abortion with SHOCK, DIC. Management planning should align with severity and response to early care, while ensuring timely specialty consultation, admission, or monitored outpatient follow-up according to risk. Typical plan statements in the source include Admit + Gynecological Review; Admit for Gynecological Review IF ACTIVELY BLEEDING If not bleeding see as OP & TCA in GOPC.

Therapeutic management should integrate supportive care with condition-specific pharmacologic treatment. Supportive measures commonly documented include See appendix (x) for hypovolemic shock resuscitation; Reassure patient, Start normal saline if actively bleeding as you consult. Treatment options listed in the source include Tranexamic acid 1g IV tds, IV Ceftriaxone 50mg/kg, IV Metronidazole 7.5mg/kg; OUT PATIENT:Tranexamic acid 1g PO tds, OP Cefuroxime 50mg/kg,PO Metronidazole 7.5mg/kg, IN PATIENT Tranexamic acid 1g IV tds IV Ceftriaxone 50mg/kg, IV Metronidazole 7.5mg/kg. Formulary examples referenced in the workbook include Hemsamic 50mg inj, Rocephin 1gm,Flagyl infusion; Hemsamic 500mg caps, Axacef 500mg tab, Flagy 400mg tablets. Final treatment choice should always be individualized to age, comorbidity profile, contraindications, pregnancy status where applicable, and observed clinical trajectory after initial intervention.

Condition 2: hotness of body

Patients presenting with this condition should first undergo a structured clinical assessment that establishes physiological stability, severity, and immediate risk. Common presentations documented in the source material include Hotness of Body. Important clinical features include No other sign; Hallucinations/delusions/abnormal posturing; Convulsions with no other sign; Convulsions with no other sign in not a known convulsive. Initial triage should prioritize early identification of shock, respiratory compromise, neurological deterioration, or obstetric and surgical emergencies where relevant.

Diagnostic evaluation should proceed in a focused sequence that supports pathway selection and timely intervention. Core investigations described in the clinical sheets include FHG + malaria AG; FHG + Malaria Ag+ U/E/C + RBS; FHG + Malaria Ag+ U/E/C + RBS (consider LP if no signs of raised ICP); FHG. Interpretive patterns used to refine diagnosis include Normal FHG+ Malaria negative; Granulocytosus + malaria negative; lymphocytosis+ malaria negative; hb< 10g/dl + malaria negative. Where diagnostic uncertainty remains, additional tests may be considered, including PBF+ Reticulocyte count+ HIV+GXM; LFTs + PBF; LFTs + PBF+ GXM+ reticulocyte count. This staged approach helps clinicians distinguish severe pathways from lower-risk pathways and supports safer escalation decisions.

Pathway-oriented diagnosis should then be articulated clearly in the clinical record. Documented diagnostic formulations include Acute febrile illness R/O viral, atypical infections; acute bacterial infection; acute viral infection; anaemia + acute febrile illness; Malaria. Management planning should align with severity and response to early care, while ensuring timely specialty consultation, admission, or monitored outpatient follow-up according to risk. Typical plan statements in the source include see as OUT PT ,TCA 7 days; admit patient for further work up and possible transfusion; see as an outpatient and TCA 3 days; admit patient for parentereral treatment.

Therapeutic management should integrate supportive care with condition-specific pharmacologic treatment. Supportive measures commonly documented include Expose patient , tepid sponge; Expose patient , tepid sponge , secure an IV line neck support, reassure the relatives; Expose patient , tepid sponge , secure an IV line neck support, reassure the relatives start ringers lactate as you consult. Treatment options listed in the source include no; amoxycillin+ clavulanate over 12years 375-750mg, below 12 years 25-50mg/kg/day, paracetamol 10mg/kg/dose; paracetamol 10mg/kg/dose, ceftriaxone 50mg/kg/dose, transfuse if symptomatic, ranferon; amoxycillin+ clavulanate over 12years 375-750mg, below 12 years 25-50mg/kg/day, paracetamol 10mg/kg/dose, artemether/lumefantrine per kg body weight. Formulary examples referenced in the workbook include fleming, paralief for children, panadol advance; Rocephin,paraconica 1g; fleming, paralief for children, panadol advance, ACTM 20/120. Final treatment choice should always be individualized to age, comorbidity profile, contraindications, pregnancy status where applicable, and observed clinical trajectory after initial intervention.

Condition 3: Elevated Blood Pressures in Pregnancy

Patients presenting with this condition should first undergo a structured clinical assessment that establishes physiological stability, severity, and immediate risk. Common presentations documented in the source material include ELEVATED BP >/= 140/90mmHg; BP reading Syst:140-179/90- 109 OR isolated 180/110 regardless of S/S; BP reading Syst:140-179 or 90- 109 mmHG. Important clinical features include WITH ANY OF:signs of target organ damage (Severe headache with blurring of vision, Epigastric pain, Oliguria , signs , liver tenderness; WITHOUT ANY OF:signs of target organ damage (Severe headache with blurring of vision, Epigastric pain, Oliguria , signs , liver tenderness.

Initial triage should prioritize early identification of shock, respiratory compromise, neurological deterioration, or obstetric and surgical emergencies where relevant.

Diagnostic evaluation should proceed in a focused sequence that supports pathway selection and timely intervention. Core investigations described in the clinical sheets include FHG + Urinalysis +U/E/C+LFTs + Coagulation profile; FHG +U/E/C+ LFTs. Interpretive patterns used to refine diagnosis include Normal FHG + Normal Transaminases + Normal U/E/C + Normal coagulation profile + No Proteinuria; Hb <10 g/dl + Normal Transaminases + Normal U/E/C + Normal coagulation profile+ No Proteinuria; Thrombocytopenia < 100 +Normal Transaminases + Normal U/E/C + Normal coagulation profile+ No Proteinuria; Normal FHG + Elevated Transaminases + Normal U/E/C + Normal coagulation profile+ No Proteinuria. Where diagnostic uncertainty remains, additional tests may be considered, including Obstetric U/S + Uric acid + PBF; Obstetric scan; Obstetric scan, PBF. This staged approach helps clinicians distinguish severe pathways from lower-risk pathways and supports safer escalation decisions.

Pathway-oriented diagnosis should then be articulated clearly in the clinical record. Documented diagnostic formulations include Severe Pre - Eclampsia; Gestational hypertension/Mild Pre Eclampsia; Gestational hypertension + Anaemia/ Mild Pre Eclampsia; Severe Pre eclampsia; Severe Pre eclampsia , Anaemia. Management planning should align with severity and response to early care, while ensuring timely specialty consultation, admission, or monitored outpatient follow-up according to risk. Typical plan statements in the source include Admit and call gynaecologist immidiately.

Therapeutic management should integrate supportive care with condition-specific pharmacologic treatment. Supportive measures commonly documented include reassure patient, place the patient in left lateral position, restrict fluids to 80mls per hr of ringers lactate avoid DNS or Dextrose.; reassure patient, place the patient in left lateral position.; reassure patient, place the patient in left lateral position,. Treatment options listed in the source include if BP > 160/110 mm/hg give IV hydrazaline 5mg slowly over 10 minutes , Give loading 4 GM of 20% MGSO4 over 15 minutes then continue with 1 GM hourly for 24 hours as you monitor the respiratory rate, urine out put and patellar reflexes. And continue for 24 hours after delivery whichever comes first.; Methyldopa 250 mg TID; Methyldopa 250mg TID; Methyldopa 250mgTID. Formulary examples referenced in the workbook include Apresolin 20mg inj, Nalepsin 4gm injection; Aldomet 250mg tablets; Aldomet 205mg tablets. Final treatment choice should always be individualized to age, comorbidity profile, contraindications, pregnancy status where applicable, and observed clinical trajectory after initial intervention.

Condition 4: HEAD INJURY

Patients presenting with this condition should first undergo a structured clinical assessment that establishes physiological stability, severity, and immediate risk. Common presentations documented in the source material include Trauma to the head. Important clinical features include WITH:-Convulsions, Confusion, Ottorrhoea/Rhinorrhoea, Penetrating, palpable #/Deformity, Blurring of vision, vomiting Anisocoria, Lateralizing signs , extremeties of age < 5 years > 60 years; Regardless of:-Convulsions, Confusion, Ottorrhoea/Rhinorrhoea, Penetrating, palpable #/Deformity, Blurring of vision, vomiting Anisocoria, Lateralizing signs extremeties of age < 5 years > 60 years; Regardless of: -Convulsions, Confusion, Ottorrhoea/Rhinorrhoea, Penetrating, palpable #/Deformity, Blurring of vision, vomiting Anisocoria, Lateralizing signs extremeties of age < 5 years > 60 years; WITHOUT:-Convulsions, Confusion, Ottorrhoea/Rhinorrhoea, Penetrating, palpable #/Deformity, Blurring of vision, vomiting Anisocoria, Lateralizing signs , extremeties of age < 5 years > 60 years. Initial triage should prioritize early identification of shock, respiratory compromise, neurological deterioration, or obstetric and surgical emergencies where relevant.

Diagnostic evaluation should proceed in a focused sequence that supports pathway selection and timely intervention. Core investigations described in the clinical sheets include Order an urgent CT scan Head,; Skull X-Ray. Where diagnostic uncertainty remains, additional tests may be considered, including U/E/C, RBS, LFTS FHG,Alcohol toxicity levels.; U/E/C, RBS, LFTS FHG, Alcohol toxicity levels. This staged approach helps clinicians distinguish severe pathways from lower-risk pathways and supports safer escalation decisions.

Pathway-oriented diagnosis should then be articulated clearly in the clinical record. Documented diagnostic formulations include Dx- Mild Head injury; Dx- Moderate Head injury; Dx- severe Head injury; Dx: Mild Head Injury; Dx: Moderate Head Injury. Management planning should align with severity and response to early care, while ensuring timely specialty consultation, admission, or monitored outpatient follow-up according to risk. Typical plan statements in the source include Admit and immediate surgical consult; Admit for Observation.

Therapeutic management should integrate supportive care with condition-specific pharmacologic treatment. Supportive measures commonly documented include Assess Airway , Circulation, Breathing, Disability and Exposure(ACBDE) Arrest Bleeding(If Present) , if in shock see appendix (x). Treatment options listed in the source include IV ceftriaxone 100mg/kg/dose , Paracetamol 10mg/kg/dose, Phenytoin 20mg/kg to run over 15 mins with ECG & BP monitoring, TT 0.5mg IM SAT in > 5 years old. Formulary examples referenced in the workbook include Rocephin 2gm,Paraconica 1gm, Epanutin 250mg inj, Tetenus injection. Final treatment choice should always be individualized to age, comorbidity profile, contraindications, pregnancy status where applicable, and observed clinical trajectory after initial intervention.

Condition 5: ABDOMINAL INJURY

Patients presenting with this condition should first undergo a structured clinical assessment that establishes physiological stability, severity, and immediate risk. Common presentations documented in the source material include Abdominal Injury. Important clinical features include WITH signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg; WITHOUT signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg. Initial triage should prioritize early identification of shock, respiratory compromise, neurological deterioration, or obstetric and surgical emergencies where relevant.

Diagnostic evaluation should proceed in a focused sequence that supports pathway selection and timely intervention. Core investigations described in the clinical sheets include Focused Assessment Sonography in Trauma (FAST U/S) as you consult. Where diagnostic uncertainty remains, additional tests may be considered, including FHG + U/E/C + GXM + LFTs; FHG + U/E/C + LFTs + GXM. This staged approach helps clinicians distinguish severe pathways from lower-risk pathways and supports safer escalation decisions.

Pathway-oriented diagnosis should then be articulated clearly in the clinical record. Documented diagnostic formulations include Blunt Abdominal injury with SHOCK; Penetrating Abdominal injury with SHOCK; Blunt Abdominal injury; Penetrating Abdominal injury. Management planning should align with severity and response to early care, while ensuring timely specialty consultation, admission, or monitored outpatient follow-up according to risk. Typical plan statements in the source include Admit and immediate surgical consult; Admit for observation.

Therapeutic management should integrate supportive care with condition-specific pharmacologic treatment. Supportive measures commonly documented include See appendix (x) for hypovolemic shoc resuscitation; See appendix (x) for hypovolemic shock resuscitation, DO NOT REMOVE any impinged penetrating object, if no impinged object PACK WITH STERILE GAUZE; Reassure patient, maintenanc IVF,. Treatment options listed in the source include IV ceftriaxone 50mg/kg/dose, Iv Metronidazole 7.5mg/kg/dose, Paracetamol 10mg/kg/dose; IV ceftriaxone 50mg/kg/dose, Iv Metronidazole 7.5mg/kg/dose, Paracetamol 10mg/kg/dose , TT 0.5mg IM SAT in > 5 years old. Formulary examples referenced in the workbook include Rocephin 1gm injection, Flagyl infusion, paraconica 1gm; Rocephin 1gm injection, Flagyl infusion, paraconica 1gm tetenus injection. Final treatment choice should always be individualized to age, comorbidity profile, contraindications, pregnancy status where applicable, and observed clinical trajectory after initial intervention.

Condition 6: CHEST TRAUMA

Patients presenting with this condition should first undergo a structured clinical assessment that establishes physiological stability, severity, and immediate risk. Common presentations documented in the source material include Trauma to the Chest + Difficulty in breathing.. Important clinical features include WITH signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg; WITHOUT signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg. Initial triage should prioritize early identification of shock, respiratory compromise, neurological deterioration, or obstetric and surgical emergencies where relevant.

Diagnostic evaluation should proceed in a focused sequence that supports pathway selection and timely intervention. Core investigations described in the clinical sheets include chest x-ray. Where diagnostic uncertainty remains, additional tests may be considered, including FHG, U/E/C. Abdominal scan especially if # is in 11th and 12th ribs or with associated abdominal tenderness. This staged approach helps clinicians distinguish severe pathways from lower-risk pathways and supports safer escalation decisions.

Pathway-oriented diagnosis should then be articulated clearly in the clinical record. Documented diagnostic formulations include Dx- Blunt chest trauma in shock.; Dx- Blunt chest trauma; Dx- Blunt chest trauma with pneumothorax; Dx- Blunt chest trauma with hemothorax. Management planning should align with severity and response to early care, while ensuring timely specialty consultation, admission, or monitored outpatient follow-up according to risk. Typical plan statements in the source include Admit and consult immidiately.

Therapeutic management should integrate supportive care with condition-specific pharmacologic treatment. Supportive measures commonly documented include See appendix (x) for hypovolemic shock resuscitation,give ovygen, Do Assess and manage Airway, Circulation, Breathing, Disability. As you consult .IF PENETRATING OBJECT INSITU, DO NOT REMOVE. ARREST ANY BLEEDING BY PUTTING A STERILE PAC Reassure patient, maintenance IVF,.IF PENETRATING OBJECT INSITU, DO NOT REMOVE. ARREST ANY BLEEDING BY PUTTING A STERILE PACK; See appendix (x) for hypovolemic shock resuscitation,give ovygen, Do Assess and manage Airway, Circulation, Breathing, Disability,isert a large bore carnular at 2nd intercostal space mid clavicular line,.IF PENETRATING OBJECT INSITU, DO NOT REMOVE. ARREST ANY BLEEDING BY PUTTING A STERILE PACK. Treatment options listed in the source include IV ceftriaxone 50mg/kg/dose, Iv Metronidazole 7.5mg/kg/dose, Paracetamol 10mg/kg/dose. Formulary examples referenced in the workbook include Rocephin 1gm injection, Flagy infusion, Paraconica 1gm injection. Final treatment choice should always be individualized to age, comorbidity profile, contraindications, pregnancy status where applicable, and observed clinical trajectory after initial intervention.

Condition 7: Epigastric PAIN

Patients presenting with this condition should first undergo a structured clinical assessment that establishes physiological stability, severity, and immediate risk. Common presentations documented in the source material include EPIGASTRIC PAIN. Important clinical features include bloody Vomitus WITH signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg; bloody Vomitus WITHOUT signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg; Non Bloody VOMITUS, WITH signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg; Non bloody Vomitus WITHOUT signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg. Initial triage should prioritize early identification of shock, respiratory compromise, neurological deterioration, or obstetric and surgical emergencies where relevant.

Diagnostic evaluation should proceed in a focused sequence that supports pathway selection and timely intervention. Core investigations described in the clinical sheets include FHG + U/E/C + LFTs + H- Pylori Ag; FHG+ H- Pylori Ag. Interpretive patterns used to refine diagnosis include Normal FHG + Normal Transaminases + Normal U/E/C + H-Pylori -VE; Hb <10 g/dl + Normal Transaminases + Normal U/E/C+ H-Pylori -VE; Thromcytopenia < 100+ Normal Transaminases + Normal U/E/C + H-Pylori -VE; Normal FHG + Elevated Transaminases + Normal U/E/C + H-Pylori -VE. Where diagnostic uncertainty remains, additional tests may be considered, including Endoscopy, GXM; Endoscopy ,coagulation profile, GXM; Endoscopy ,coagulation profile, Abdominal ultrasound, GXM. This staged approach helps clinicians distinguish severe pathways from lower-risk pathways and supports safer escalation decisions.

Pathway-oriented diagnosis should then be articulated clearly in the clinical record. Documented diagnostic formulations include probable dx- Upper GI bleeding. With shock; probable dx- Upper GI bleeding, Anaemia With shock; Probable dx- Upper GI bleeding with shock R/O a coagulopathy; probable dx- Liver failure, oesaphageal varices.; probable dx- Liver failure, oesaphageal varices. Coagulopathy.. Management planning should align with severity and response to early care, while ensuring timely specialty consultation, admission, or monitored outpatient follow-up according to risk. Typical plan statements in the source include Admit and urgent surgical consult; assess, see as an OPD and a TCA of 7 days.

Therapeutic management should integrate supportive care with condition-specific pharmacologic treatment. Supportive measures commonly documented include See appendix (x) for hypovolemic shock resuscitation; start IV normal saline as you consult; start on pain relieve. Treatment options listed in the source include start on fluid resuscitation (see appendix x), iv clarithromycin ,1 month - 12 years 7.5mg/kg BID DO NOT EXCEED 500MG > 12 years 500mg BID IV Augmentin < 12 years 25mg/kg/day TID >12 years 45mg/kg TID IV Ranitidine 50mg QID > 16 Years > 16 years 2mg /kg /day TID ,Trenexamic 10mg/kg/dose/TID; Start on, iv clarithromycin ,1 month - 12 years 7.5mg/kg BID DO NOT EXCEED 500MG > 12 years 500mg BID IV Augmentin < 12 years 25mg/kg/day TID >12 years 45mg/kg TID IV Ranitidine 50mg QID > 16 Years > 16 years 2mg /kg /day TID ,Tranexamic acid 10mg/kg/dose TID; start on fluid resuscitation (see appendix x), iv clarithromycin ,1 month - 12 years 7.5mg/kg BID DO NOT

EXCEED 500MG > 12 years 500mg BID IV Augmentin < 12 years 25mg/kg/day TID >12 years 45mg/kg
Ranitidine 50mg QID > 16 Years > 16 years 2mg /kg /day TID ,; > 12 years esomeprazole 20mg BID /da
, paeds 3.5kg : 2.5mg P.O /day , 3.5kg-7.5kg: 5mg PO /day, > 7.5 kg 10mg PO /day. Formulary exampl
referenced in the workbook include Klacid 500mg injection,Augmentin 1.2gm injection, Zantac 50mg inj, Hemsamic 500mg injection; Klacid 500mg injection,Augmentin 1.2gm injection, Zantac 50mg inj; Klacid 500mg injection,Augmentin 1.2gm injection, Zantac 50mg inj, hemsamic 500mg inj. Final treatment choice should always be individualized to age, comorbidity profile, contraindications, pregnancy status where applicable, and observed clinical trajectory after initial intervention.

Condition 8: WHEEZE

Patients presenting with this condition should first undergo a structured clinical assessment that establishes physiological stability, severity, and immediate risk. Common presentations documented in the source material include Wheezing with Difficulty in Breathing. Important clinical features include WITH ANY OF: While walking, Talks in Alertness, Can lie down, SentencesMay be agitated, Increased Respiratory rate,no use of accessory muscles,wheeze often only end expiratory, Pulse Rate <100, Pulsus paradoxus, Absent/ <10 mm Hg; WITH ANY OF: While at rest (infant—softer, shorter cry, difficulty feeding),Commonly uses accessory muscles, wheeze Loud throughout exhalation,Pulse Rate 100-120 Age 2 -12Mo <160/min, 1-2 Years <120/min 2-8 years < 110/min, Pulsus paradoxus May be present 10–25 mm Hg; WITH ANY OF:While at rest (infant—stops feeding), Sits upright, Words Usually agitated, wheeze loud throughout inhalation and exhalation,Pulse Rate >120, Pulsus paradoxus, Often present . 25 mm Hg (adult), 20–40 mm Hg (child); WITH: Drowsy / Confused, paradoxical thoracoabdominal movement,absence of wheeze, Bradycardia, absence of pulsus paradoxus. Initial triage should prioritize early identification of shock, respiratory compromise, neurological deterioration, or obstetric and surgical emergencies where relevant.

Diagnostic evaluation should proceed in a focused sequence that supports pathway selection and timely intervention. Core investigations described in the clinical sheets include FHG; FHG + U/E/C; FHG + U/E/C + BGAs. This staged approach helps clinicians distinguish severe pathways from lower-risk pathways and supports safer escalation decisions.

Pathway-oriented diagnosis should then be articulated clearly in the clinical record. Documented diagnostic formulations include MILD ASTHMATIC ATTACK; MODERATE ASTHMATIC ATTACK; SEVERE ASTHMATIC ATTACK; ACTUAL/ IMPENDING RESPIRATORY FAILURE. Management planning should align with severity and response to early care, while ensuring timely specialty consultation, admission, or monitored outpatient follow-up according to risk. Typical plan statements in the source include TREAT AS OP & TCA 24HRS,; ADMIT & CALL PHYSICIAN IMMEDIATELY; ADMIT IN CRITICAL CARE & CALL PHYSICIAN IMMEDIATEL

Therapeutic management should integrate supportive care with condition-specific pharmacologic treatment. Supportive measures commonly documented include Reassure patient/ guardian, administer oxygen to maintain SPO2 > 90%, Give health education See appendix (XII); INTUBATE PATIENT Reassure patient/ guardian, administer oxygen to maintain SPO2 > 90%, Give health education See appendix (XII). Treatment options listed in the source include Nebulize with Salbutamol 1Mo - 11 yrs 2.5mg, > 11 years 5mg can be repeated up to 3 doses in the 1st hour + Predinisolone 1mg/kg daily for 1/52; Nebulize with Salbutamol 1Mo - 11 yrs 2.5mg, > 11 years 5mg can be repeated up to 3 doses in the 1st hour + IV Ipratropium 200mg + IV Hydrocortisone 50mg(1-5 yrs), 100mg (6-12 yrs),. Formulary examples referenced in the workbook include Ventolin nebulizing solution, prednisolone 5mg tablets; Ventolin nebulizing solution, Atrovent 250mg nebules, Hydrocortison 100mg inj. Final treatment choice should always be individualized to age, comorbidity profile, contraindications, pregnancy status where applicable, and observed clinical trajectory after initial intervention.

Condition 9: BURNS

Patients presenting with this condition should first undergo a structured clinical assessment that establishes physiological stability, severity, and immediate risk. Common presentations documented in the source material include PATIENT WITH BURNS. Important clinical features include Partial thickness burns greater than 10% total body surface area (TBSA). 2. Burns that involve the face, hands, feet, genitalia, perineum, or major joints. 3. Third degree burns in any age group. 4. Electrical burns, including lightning injury. 5. Chemical burns. 6. Inhalation injury. 7. Burn injury in patients with preexisting medical disorders that could complicate management, prolong recovery, or affect mortality. 8. Any patient with burns and concomitant trauma (such as fractures) in which the burn injury poses the greatest risk of morbidity or mortality. . 9. Extremes of age <5 & >60 10. Burn injury in patients who will require special social, emotional, or rehabilitative intervention 12. Circumfrencial burns. Initial triage should prioritize early identification of shock, respiratory compromise, neurological deterioration, or obstetric and surgical emergencies where relevant.

Diagnostic evaluation should proceed in a focused sequence that supports pathway selection and timely intervention. This staged approach helps clinicians distinguish severe pathways from lower-risk pathways and supports safer escalation decisions.

Pathway-oriented diagnosis should then be articulated clearly in the clinical record. Documented diagnostic formulations include MILD BURN INURY; MODERATE BURN INJURY; SEVERE BURN INJURY. Management planning should align with severity and response to early care, while ensuring timely specialty consultation, admission, or monitored outpatient follow-up according to risk. Typical plan statements in the source include Treat as OP & TCA 3 days; Admit for Surgical review.

Therapeutic management should integrate supportive care with condition-specific pharmacologic treatment. Supportive measures commonly documented include IN SEVERE BURNS DO ACBE remove all burned clothing, immerse the site in cold water for 30 minutes,douse with cool water, apply clean wraps on burned area,administer tetanus prophylaxis, gently cleanse the burn with 0.25% chlorhexidine solution, DO NOT use alcohol-based solutions,Gentle scrubbing will remove the loose necrotic tissue. Apply a thin layer of antibiotic cream (silver sulfadiazine). Dress the burn with petroleum gauze and dry gauze; IN SEVERE BURNS DO ACBE remove all burned clothing, immerse the site in cold water for 30 minutes,douse with cool water, apply clean wraps on burned area,administer tetanus prophylaxis, gently cleanse the burn with 0.25% chlorhexidine solution, DO NOT use alcohol-based solutions,Gentle scrubbing will remove the loose necrotic tissue. Apply a thin layer of antibiotic cream (silver sulfadiazine). Dress the burn with petroleum gauze and dry gauze SEE APPENDIX 12 FOR FLUID MANEGEMENT IN BURNS. Treatment options listed in the source include Fluloxacillin 50mg/kg/dose + Paracetamol 10mg/kg/dose + silver sulphur diazine cream; IV Fluloxacillin 50mg/kg/dose + IV Metronidazole 7.5mg/kg + Pethidine 50mg IM + Tetracycline eye ointiment in facial burns, Pethidine 50mg IM,Omeprazole 4mg/kg/dose,silver sulphur diazine cream; IV Fluloxacillin 50mg/kg/dose + IV Metronidazole 7.5mg/kg + Pethidine 50mg IM + Tetracycline eye ointiment in facial burns, Pethidine 50mg IM,Omeprazole 4mg/kg/dose, silver sulphur diazine cream. Formulary examples referenced in the workbook include Adult Floxapen 500mg caps, Panadol advance, silverex 500gm, 25gm, Paed floxapen 125mg syrup, panadol baby and infant, silverex 500gm, 25gm; Floxapen 500mg inj, Flagyl infusion, pethidine 50mg, tetracycline eye ointment,losec 40mg injection,silverex cream 500gm. Final treatment choice should always be individualized to age, comorbidity profile, contraindications, pregnancy status where applicable, and observed clinical trajectory after initial intervention.

Condition 10: HYPONATREMIA

Patients presenting with this condition should first undergo a structured clinical assessment that establishes physiological stability, severity, and immediate risk. Initial triage should prioritize early identification of shock, respiratory compromise, neurological deterioration, or obstetric and surgical emergencies where relevant.

Diagnostic evaluation should proceed in a focused sequence that supports pathway selection and timely intervention. This staged approach helps clinicians distinguish severe pathways from lower-risk pathways and supports safer escalation decisions.

Pathway-oriented diagnosis should then be articulated clearly in the clinical record. Management planning should align with severity and response to early care, while ensuring timely specialty consultation, admission, or monitored outpatient follow-up according to risk.

Therapeutic management should integrate supportive care with condition-specific pharmacologic treatment. Treatment options listed in the source include (Serum Na < 130 mmol/L); signs and symptoms; nausea,malaise,lethergy,decreased level of conciousness, headache,if severe(seizures and coma),overt neurologic symptoms (<115 mEq/L); CLASSIFICATION. Final treatment choice should always be individualized to age, comorbidity profile, contraindications, pregnancy status where applicable, and observed clinical trajectory after initial intervention.

End of Handbook